

people in Bengaluru, Pune, Chennai, and Gurugram, working across software engineering, AI development, global manufacturing, and operations.

One major focus is improving staff efficiency and reducing burnout, especially among nurses and clinicians. We have studied hospital workflows to develop systems that reduce time spent on administrative or repetitive tasks. In intensive care units, for instance, nurses often monitor multiple patients simultaneously. Our technologies highlight only exceptions or critical alerts, enabling staff to prioritise care over manual data entry. We are also applying AI in imaging, particularly MRI, to reduce scan times and improve quality. AI tools now correct for motion artifacts and minimise repeat scans, reducing overall duration from 45 minutes to 20–25 minutes in many cases.

We continue to expand digital solutions such as remote diagnostics and telehealth—crucial for

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underserved areas. Our focus remains on delivering innovations that ease clinical workloads and improve patient outcomes. Our investments will reinforce these goals both in India and globally.

Q: What are some of the key trends you are observing in med-tech or healthcare in India?

A: Three major shifts are reshaping India’s healthcare and med-tech landscape. The first is the need to do more with existing assets. India boasts highly skilled clinicians, but there is a gap in timely and accurate diagnoses. This can be addressed by investing in new equipment and improving utilisation of existing infrastructure. Artificial intelligence is pivotal here. Five more scans per day with the same machine delivers a higher return on investment. Thus, inno-

vation is increasingly about smarter utilisation, not just capital spending.

The second shift is the transition of care from hospitals to home settings. For instance, after a total knee replacement, a patient can wear sensors that send real-time data—step count, physiotherapy compliance, and even fall alerts—to the hospital via smartphone. This extends meaningful care beyond hospital walls.

The third shift is the use of technology to address staff shortages in Tier II and III cities. One solution is our remote operations control centre, where expert radiologists in metropolitan areas guide technicians in smaller towns, assisting with scan optimisation, positioning, and interpretation in real-time.

Q: How is Philips using AI in healthcare, and where does human judgement fit in?

A: AI is a powerful tool, but it must remain an enabler, not a replacement, for clinical judgement. It analyses large data sets, flags anomalies, and suggests potential diagnoses, but the final decisions must always rest with doctors.

A case in point is digital pathology. AI enables high-resolution slide scans that specialists can access online or remotely, reducing the strain and delays of traditional microscopy. This has helped clinicians avoid musculoskeletal conditions like cervical spondylosis caused by long hours at microscopes.

Q: What are the key therapy areas Philips is currently focusing on, and how are your latest innovations and product launches aligned with these priorities?

A: We are concentrating on three high-impact areas: Ischemic stroke care, cardiac ultrasound, and non-invasive cancer diagnostics.

In stroke care, we have launched the Azurion 3.0 Cath Lab system in India. Previously used mainly for cardiac procedures, it is gaining traction for neuro interventions. We are also piloting a direct-to-angio-suit approach—bypassing CT or MRI for eligible stroke patients to reduce treatment delays. In cardiac ultrasound, we have upgraded our transcend platform with AI to improve diagnoses of structural heart diseases. For end-stage renal disease patients who cannot undergo contrast imaging, we now combine MRI and ultrasound for contrast-free, safer diagnostics. We are training 300 doctors in India on this protocol.

We also collaborate globally with start-ups offering API- or app-based innovations that integrate with our artificial intelligence platforms. In India, we work closely with hospital chains to